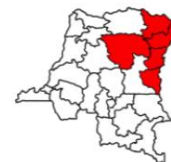




EBOLA PRESIDENTIAL FORCE TASK 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°0100/MVEBDB/08/22/2026

Reporting date: August 22, 2026

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HIGHLIGHTS (last 24 hours)

As of August 22, 2026, there were 55 new confirmed cases of Ebola Virus Disease, including 23 community deaths.

Ebola cases (EVs) have been recorded in the provinces of Ituri (35 cases), North Kivu (16 cases), and Haut-Uélé (4). No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 5,514	ACCUMULATIVE DEATH CONFIRMED 2,642	LETHALITY 47.9%	PATIENTS IN ISOLATION/CTE 808	ACCUMULATIVE HEALED 1,200	TRACKING RATES CONTACTS (Of the Day) 83.4%
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo & Bas Uélé



57 out of 151 health zones affected (37.7%)



Summary of key points from the last 24 hours

In the past 24 hours, **55 new confirmed cases**, including **23 community deaths** from Ebola Virus Disease (EVD), have been reported in the provinces of **Ituri (35 cases)**, **North Kivu (16)** and **Haut Uélé (4)**.

Over the past 24 hours, **18 patients** have been declared recovered after receiving two negative Ebola virus (EV) test results (14 in Ituri, 3 in North Kivu, and 1 in Tshopo). On the other hand, **13 deaths within Ebola treatment centers** have been recorded, including 10 in Ituri and 3 in North Kivu.

From the start of the epidemic until August 22, 2026, **5,514 confirmed cases, including 2,642 deaths**, were reported in the DRC, representing a case fatality rate of **47.9 %**. Ituri remains the epicenter of the epidemic, accounting for 83.6% of cumulative confirmed cases and 63.6% of new cases reported in the last 24 hours.

Overall, 57 of the 151 health zones across the six provinces are affected.

since the start of the epidemic in the DRC. Ituri has 28 health zones affected out of 36, followed by North Kivu (13/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas Uélé (2/11).

Distribution of confirmed cases and deaths by affected province

(As of August 22, 2026)

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	4607	2065	44.8%	28/36 (77.8%)	35
North Kivu	714	490	68.6%	13/34 (38.2%)	16
Haut-Uélé	173	77	44.5%	6/13 (46.1%)	4
Tshopo	15	8	53.3%	7/23 (30.4%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Bas Uélé	2	1	50.0%	2/11 (18.2%)	0
Total	5,514	2,642	47.9%	57/151 (37.7%)	55

*An additional confirmed case has been added to the total for Haut-Uélé following a data correction/reconciliation, bringing the national total to 5,514 cases. This case is not included in the 55 new confirmations of August 22nd.

Confirmed cases and deaths by province and health zone (situation as of August 22, 2026)

Cumulative number

Current situation (24 hours)

Province / Health Zone	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	4607	2065	44.8%	35	12	10	22
Adja	11	1	9.1%				0
Aru	7	6	85.7%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	127	28	22.0%	1	0		0
Boga	1	1	100.0%				0
Bunia	1279	402	31.4%	17	7		7
Damascus	27	9	33.3%				0
Drodro	13	6	46.2%				0
Fataki	73	30	41.1%				0
Gethy	5	2	40.0%				0
Kambala	2	0	0.0%				0
Kilo	31	12	38.7%				0
Komanda	58	38	65.5%	1	1		1
Lita	196	108	55.1%	4	2		2
Logo	11	5	45.5%				0
Lolwa	17	4	23.5%				0
Mahagi	4	1	25.0%				0
Mambasa	14	7	50.0%				0
Mandima	32	19	59.4%	2	0		0
Mangala	191	106	55.5%				0
Mongbwalu	600	283	47.2%				0
Nia-Nia	191	107	56.0%	1	0		0

Nizi	609	252	41.4%	6	2		2
Nyankunde	122	35	28.7%				0
Rimba	10	4	40.0%				0
Rwampara	902	334	37.0%	2	0		0
Tchomia	54	25	46.3%	1	0		0
To ventilate	N / A	233	N / A	N / A	N / A	10	10
North Kivu	714	490	68.6%	16	11	3	14
Blessed	114	84	73.7%	6	3		3
Butembo	137	106	77.4%	1	1	2	3
Goma	1	0	0.0%				0
Kalunguta	13	6	46.2%				0
Katwa	342	229	67.0%	8	6	1	7
Kyondo	16	12	75.0%				0
Lubero	1	1	100.0%				0
Mabalako	3	2	66.7%	1	1		1
Masereka	7	4	57.1%				0
Musienene	68	39	57.4%				0
Mutwanga	1	1	100.0%				0
Oicha	6	4	66.7%				0
Vuhovi	5	2	40.0%				0
Haut-Uélé	173	77	44.5%	4	0	0	0
Boma	24	9	37.5%				0
Mangbetu	3	1	33.3%				0
Gombari	3	1	33.3%				0
Isiro	67	28	41.8%	4	0	0	0
Pawa	20	15	75.0%				0
Rungu	1	0	0.0%				0
Wamba	58	24	41.4%				0
Tshopo	15	8	53.3%	0	0	0	0
Bafwasende	1	0	0.0%				0
Kabondo	3	1	33.3%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	6	4	66.7%				0
Mangobo	2	2	100.0%				0
Tshopo	1	0	0.0%				
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	2	1	50.0%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	1	0	0.0%				
Total	5514	2642	47.9%	55	23	13	36

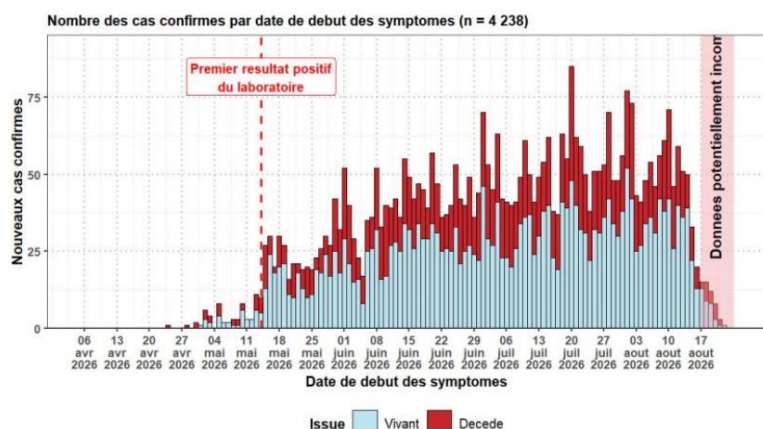
*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

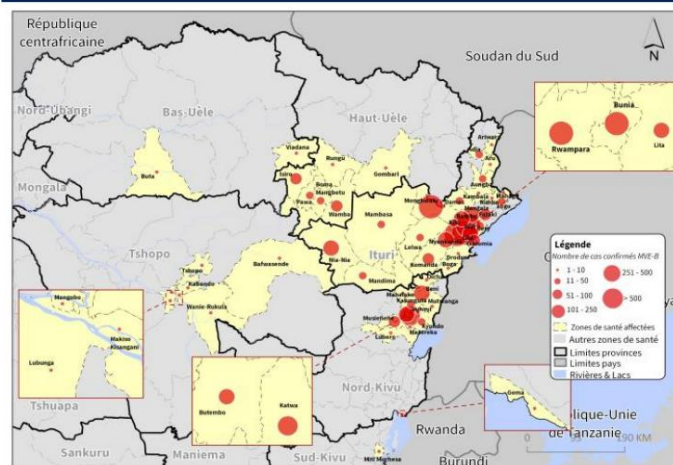
Status of alerts notified by province, as of August 22, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	16	1	17	14	1	2	0	15	14
Bas Uélé	6	0	6	6	0	0	0	2	1
Ituri	252	22	274	54	20	178	1	64	43
North Kivu	693	49	742	62	49	620	0	111	62
South Kivu	5	0	5	2	0	3	0	2	2
Tshopo	120	0	120	4	0	93	0	4	1
Total General	1092	72	1,164	142	70	896	1	198	123

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- **North Kivu** : participation in the joint monitoring and evaluation mission of the response in the health zones of Katwa, Musienene and Butembo; launch of training for local supervisors in Butembo and the Beni pool (health zones of Beni, Mabalako and Kalunguta) with the support of UNICEF.
- **Ituri**: supervision of response activities and holding of the coordination meeting.
- **Tshopo** : meeting with chief medical officers of coordination and technical and financial partners; continuation of construction work on the isolation units of Madula and Kabondo as well as the large capacity CTE.
- **Bas-Uélé**: Coordination meeting held with chief medical officers and partners technical and financial.

1.1.2. Epidemiological surveillance

1.1.3. Main Actions

At the end of the day on August 22, 2026, 1,164 alerts were recorded, including 1,109 (95.3%) were verified in the six affected provinces. Following these verifications, 212 (19.1%) were validated as suspected cases and 198 (93.4%) were investigated, of which 123 were transferred to the CT/CTE.

The proportion of contacts monitored in the last 24 hours has risen to 83.4% (14,277 seen out of 17,127 to be monitored), below the 85% threshold: Ituri 85.8% (5,954/6,940), Tshopo 100% (490/490), and North Kivu 82.2% (6,448/7,846). Haut-Uélé 77.9% (1,260/1 617) and Bas Uélé 53.4% (125/234) show low follow-up rates.

1.1.4. Challenges

Weak follow-up of contacts around confirmed cases mainly in the provinces of Haut Uélé and Bas Uélé linked to a lack of teams dedicated to these activities and insufficient completeness in Ituri (8 health zones did not transmit data on contact follow-up).

1.2. Update on checkpoint and point of entry (PoC/PoE) activities

1.2.1. Main actions

- Three (3) alerts were notified to the PoE/PoC in the last 24 hours: two in Ituri, both validated and one transferred to the CT/CTE (50%), and one in North Kivu (PoC Kasitu, AS Bundji, ZS of Beni), where a lifeless body was intercepted and collected.

Indicators	Ituri	North-Kivu	South-Kivu	Tshopo Haut-Uele	Down-Uele	Overall	
Report completeness	98.3% (59/60)	100% (18/18)	55.5% (5/9)	77.3% (17/22)	45% (5/11)	42.1% (8/19)	80.6%
% of functional strategic PoE/PoC	9.09% (1/11)	N / A	N / A	N / A	N / A	57% (8/14)	36.0% (9/25)
Number of people who used PoE/PoC	130 388 91	398 8 936		6,664	9080	ND 246 466	
% of travelers screened % of travelers who washed their hands	98.0% 96.9%	98.5% 100%	95.0% ND 97.6%				
	96.0% 96.9%	98.5% 99.5%	85.0% ND 96.1%				
% of travelers made aware: 99.4% 96.9%	100% 100%	94.7% ND 98.3%					
Number of alerts notified	02	1	0	0	0	0	3
Number of validated live alerts	02	0	0	0	0	0	2
Number of bodies intercepted today; % of suspected cases	0	1	0	0	0	0	1
transferred to CT/CTE	50% (1/2)	0% 0% 0%	0% 0%			0%	50% (1/2)
% of lifeless bodies swabbed/secured	0% 100%	0% 0% 0%				0% 100%	
% of suspected cases investigated within 24 hours	100% 100%	0% 0% 0%				0% 100%	

1.2.2. Challenges

Insufficient operationalization of PoE/PoC: report completeness remains low in Haut-Uélé (5 out of 11 reports; 45.4%), Sud-Kivu (5 out of 9; 55.5%), and Tshopo (17 out of 22; 77.3%); in Ituri, only one of the 11 strategic PoE/PoCs is operating without interruption (9.1%). Five PoCs in Haut-Uélé remain on strike due to non-payment of service providers, and 2,820 travelers (3%) refused screening in Nord-Kivu.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 35 new positive results** (23 living and 12 deaths) out of 173 new samples received and analyzed (**positivity: 20.2 %**).
- **North Kivu : 16 new positive results** (5 living and 11 deaths) out of 140 samples analyzed (**positivity of 11.4 %**).
- **Haut-Uélé: 4 new positive results** (4 living and 0 deaths) out of 11 samples analyzed (**positivity of 36.4%**).
- **South Kivu** : 2 new samples received and analyzed in the Miti-Murhesa health zone, all negatives
- **Bas-Uélé** : 1 sample received and analyzed in the Buta health zone, negative.

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 14 rings were opened out of the 57 identified (24.6%), 24 households decontaminated out of 51 identified (47.1%) and 7 out of 7 FOSA (100%); 5 PCI kit allocations in Bunia and 5 FOSA assessments were carried out.
- **In North Kivu**, 5 FOSA were monitored and supported, 375 providers briefed and 256 handwashing devices monitored; 5 rings were opened, 12 households and 3 FOSA were decontaminated; training of local supervisors in Butembo and the Beni pool (ZS of Beni, Mabalako and Kalunguta) was launched with the support of UNICEF.
- **In Bas-Uélé**, 9 of the 13 targeted ESSs were accompanied, 11 ESSs remain under PCI surveillance and PCI kits were delivered to Buta and Ganga; the decontamination of the household of the confirmed case in Viadana was carried out.
- **In Tshopo**, 12 ESS were accompanied in 3 health zones, where 69 providers were briefed (Makiso-Kis: 3 ESS and 21 providers; Kabondo: 8 ESS and 41 providers); the health post of the 3rd Defense Zone (Makiso-Kis) was decontaminated (10 premises, 3 latrines and 3 showers) and the TDRs of the training of 85 hygienists from 5 health zones validated; around the 15th case, 1 ring was opened, the FOSA and the household decontaminated and a kit distributed.
- **In Haut-Uélé**, 3 infection control rings were opened in Isiro, 3 households and 2 health establishments were decontaminated; 29 health establishments remain under infection prevention and control (IPC) surveillance, 3 out of 5 planned health establishments were assessed, 8 out of 10 received support, and 31 service providers and hygiene workers were briefed out of 40 expected. Of the 3 death alerts received (1 in Wamba and 2 in Pawa) and 1 report from the previous day, 2 bodies were recovered by swab (1 failed), and 3 blood donations were carried out by the Red Cross team.

1.4.2. Challenges

- Poor implementation of IPC standards: in Ituri, ring opening remains limited to 24.6% due to insufficient decontamination teams (Fataki, Mangala), lack of mobility (Komanda, Bambu), and supply shortages in Mandima; 12 swab failures and

Resistance to EDS was recorded in Bambu, Nyakunde, Rwampara, Aru and Fataki.

In North Kivu, only 7 out of 12 health zones have been fully equipped, and no IPC kits have been distributed. In Bas-Uélé, decontamination of the household of relatives of the first confirmed case in Buta could not be carried out due to resistance. In Tshopo, activities are blocked in Bafwasende for security reasons.

1.5. Continuity of care

1.5.1. Main actions

- **In Ituri**, 94 new admissions were recorded, with 87 discharges, including 14 recoveries and no escapees. Thus, 528 patients are hospitalized in 965 beds, representing an overall occupancy rate of 54.7%.
- **In North Kivu**, 19 admissions were recorded (5 confirmed and 14 suspected) with 28 discharges, including 3 recoveries at the Beni Ebola Treatment Center. Thus, 167 patients are hospitalized out of 206 beds, representing an occupancy rate of 81.1%.
- **In Tshopo**, a new admission of a suspected case to the CTE HC Kisangani and a discharge of recovery bring to 4 the number of isolated patients (1 confirmed and 3 suspected) for 14 beds, or 28.6% occupancy.
- **In Haut-Uélé**, 11 admissions were recorded (4 confirmed and 7 suspected). Thus, 72 Patients are hospitalized in 130 beds, representing 55.4% occupancy.
- **In South Kivu**, 29 suspected cases remain hospitalized.

1.5.2. Challenges

Limited capacity and inadequate technical facilities at some Ebola Treatment Centers (ETCs): In Ituri, the ETCs in Nizi and Bambu (confirmed cases) and Kilo, Logo, and Kigonze (suspected cases) are overcrowded, with 25 patients (Kilo 6, Sota 6, Lolwa 9, Rimba 1, and Mandima 3) receiving no food assistance. In North Kivu, several suspected cases in Butembo, Masereka, Vuhovi, and Kalunguta are isolated in specialized health centers (ESS) that are not transit centers. In Haut-Uélé, the seven operational ETCs remain insufficiently equipped, and occupancy exceeds capacity in Isiro (120% confirmed cases) and Wamba (140% suspected cases); in Bas-Uélé, the construction of the Buta ETC must be expedited, and no ambulances are available.

1.6. Risk communication and community engagement

1.6.1. Main actions

- **In Ituri**, 236 contacts were listed around confirmed cases (164 around 11 active cases in the Kilo health zone and 72 around 8 positive cases in the Rwampara health zone); 3 EDS were facilitated (1 in Kilo and 2 in Rwampara), 3 households and 1 structure decontaminated, 8 positive results announced to families and 3 recovered patients accompanied and reintegrated in Rwampara; 22 broadcasts and 6 radio spots were produced, 36 community radio stations engaged and 42 IEC materials distributed.
- **In Tshopo**, two rumors were clarified and 1,241 people were sensitized during home visits in 212 households in Makiso and Mangobo; authorities and leaders of the Bakumu Mandombe sector (Wanie-Rukula) were sensitized during an expanded security committee meeting with 86 participants.
- **In North Kivu**, the 3 recovered patients from the CTE at the Beni General Referral Hospital were discharged and accompanied to their health areas; 20 new influencers from Butembo were briefed on community incidents during the EDS; a community dialogue was held with the heads of groups in the Baswagha chiefdom (Musienene Health Zone); 11 cases covered, 85 household heads affected and 15 rumors clarified.

1.1.1. Challenges

- Persistent community challenges: In Ituri, weak involvement of traditional authorities and leaders in the Mangala (Tchere Health Zone) and Bunia (Babao Bokoe community) health zones; demotivation of community health workers in Nia-Nia due to lack of payment; and 5 incidents of reluctance in Kilo, 3 of which have been resolved; only 8 of the 28 affected health zones in Ituri and 2 in North Kivu reported community concerns, representing 19%. In North Kivu, neither of the 2 community incidents in Beni and Kyondo has been resolved, and a rumor of Ebola virus disease (EVD) being injected into children is hindering the measles-rubella-polio campaign in the Ngeleza Health Zone.

1.7. Logistics

1.7.1. Main actions

- **In Ituri**, 236 contacts were listed around confirmed cases (164 around 11 active cases in the Kilo health zone and 72 around 8 positive cases in the Rwampara health zone); 3 EDS were facilitated (1 in Kilo and 2 in Rwampara), 3 households and 1 structure decontaminated, 8 positive results announced to families and 3 recovered patients accompanied and reintegrated in Rwampara; 22 broadcasts and 6 radio spots were produced, 36 community radio stations engaged and 42 IEC materials distributed.
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1.8. Security

1.8.1. Main actions

- **In North Kivu**, the pillar continued to provide security support to EDS and decontamination teams, in a context of community resistance reported in Katwa, Butembo, Kalunguta and Beni and the impassability of the road leading to the Bundji cemetery (Beni health zone).
- **In Haut-Uélé**, the response teams were attacked in Isiro and Pawa; the pillar continued its security and public relations activities with the deployed teams.

1.8.2. Challenges

In Ituri, a response team was chased away with machetes during an attempt at EDS in the Kilo SZ, where 5 incidents of reluctance were recorded, of which 3 were resolved and one refusal of EDS was reported.

In North Kivu, access to the household of a confirmed case of AS in Butsili (ZS of Beni) was blocked, preventing the announcement of the result, and the road leading to the Bundji cemetery remains impassable.

1.9. PSEA (Prevention of Sexual Exploitation and Abuse)

1.9.1. Main actions

Continued support for training of community, military and police leaders on PSEA and MVE, jointly with the communications pillar.

1.9.2. Challenges

- Still insufficient coverage of PSEA activities, limited to 4 out of 36 health zones in Ituri, lack of mapping of stakeholders, insufficient mass awareness materials and lack of transport and computer equipment for the pillar; Coverage remains limited to 4 of the 36 health zones of Ituri (11.1%) and only two partners (WHO and Africa CDC) report their PSEA activities.

KEY MESSAGES

- Ebola virus transmission remains active, with 55 new cases including 23 community deaths and 13 intra-CTE recorded in the last 24 hours, bringing the national total to 5,514 confirmed cases and 2,642 deaths.
- Ituri remains the epicenter of the epidemic, concentrating 83.6% of cumulative confirmed cases and 63.6% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).
- Surveillance shows good performance (93.4%) of suspected cases investigated while the follow-up rate (83.4%) is close to the threshold of 85.0%.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of 64 support teams.



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